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Research and analysis

4. Children and young people

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Contents

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Introduction

This is Chapter 4 of the [COVID-19 mental health and wellbeing surveillance report (https://www.gov.uk/government/publications/covid-19-mental-health-and-wellbeing-surveillance-report)](https://www.gov.uk/government/publications/covid-19-mental-health-and-wellbeing-surveillance-report).

This is the final version of this regularly updated chapter. It includes evidence and analysis that was released into the public domain up to 25 January 2022. After this release it will not be updated again.

The chapter presents emerging findings from UK studies of the mental health and wellbeing of children and young people (CYP) in relation to the coronavirus (COVID-19) pandemic.

It presents a high-level summary of the best, recent, evidence available about the experience of children and young people of the pandemic as relevant to understanding their mental health and wellbeing. It is based on a range of evidence sources of differing methods and quality. Studies reported include longitudinal data. Some have representative samples, others have convenience samples. Study detail is included with each reference - found at the bottom of the section.

As such, many of the findings presented below need to be considered as indicative and not conclusive evidence of impacts at this stage due to these methodological constraints. Nevertheless, they provide an indication of the experiences of children and young people in the recent period and the groups at greater risk of experiencing poor mental health or greater impacts on their mental health.

Details of the method of searching and compiling evidence can be found in the [Methodology document (https://www.gov.uk/government/publications/covid-19-mental-health-and-wellbeing-surveillance-report/methodology)](https://www.gov.uk/government/publications/covid-19-mental-health-and-wellbeing-surveillance-report/methodology).

> Note: This chapter presents evidence on reported experiences, reported wellbeing and the reported symptoms of mental health. Any deterioration of mental health captured in studies and self-reported surveys and weekly reporting should not be automatically interpreted as an increase in mental illness or need for mental health services.

Important gaps in publicly available findings so far include:

- changes in children’s mental health against a recent pre-COVID-19 baseline
- representative evidence on the experiences and mental health of children and young people from particular sub-groups, such as young lesbian, gay, bisexual, and transgender (LGBT) people and those in ethnic minority groups
- experiences of important pandemic related risk such as illness or bereavement of close family relatives

Important findings

Evidence suggests that some children and young people’s mental health and wellbeing has been substantially impacted during the pandemic.

Between March and June 2020, a period when schools were closed to most pupils, symptoms of depression and post traumatic stress disorder (PTSD) were found to have significantly increased in children and young people aged between 7.5 and 12 years old compared to immediately before the pandemic (references 1 to 2). However, some pupils reported sleeping and feeling better (https://psyarxiv.com/gtbfm/) and primary aged children were more likely to report that they felt happier and less lonely during the lockdown than secondary aged pupils. During these months, some parents and carers reported an overall increase in mental health problems in their children (references 4 to 5).

By September 2020, relative to the March to June 2020 lockdown, reported behavioural, attention, and emotional difficulties in children had returned to, and stabilised at, a lower level (http://cospaceoxford.org/findings/changes-in-childrens-mental-health-symptoms-from-march-2020-to-june-2021/).

The Department for Education’s (DfE’s) State of the Nation (https://www.gov.uk/government/publications/state-of-the-nation-2021-children-and-young-peoples-wellbeing) - an annual report that draws upon a range of sources on children and young people’s mental health, wellbeing, and experiences over the previous academic year - found that evidence indicated lower wellbeing in December 2020 and February 2021, when schools were closed to most pupils, compared to previous months in the academic year. Reductions in average levels of wellbeing occurred most clearly in February 2021, when schools were closed to the majority of children, before recovering towards the end of the academic year as restrictions were eased.

Data from February and March 2021 shows that rates of probable mental disorder in children and young people have increased between 2017 and 2021 (https://digital.nhs.uk/data-and-information/publications/statistical/mental-health-of-children-and-young-people-in-england/2021-follow-up-to-the-2017-survey) (rates identified in 2020 were similar to 2021). In 6 to 16 year olds, rates had increased from 11.6% to 17.4%, among 17 to 19 year olds, rates had increased from 10.1% to 17.4%. Additionally, the proportion of children and young people with possible eating problems also increased (https://digital.nhs.uk/data-and-information/publications/statistical/mental-health-of-children-and-young-people-in-england/2021-follow-up-to-the-2017-survey). Among 11 to 16 year olds, the proportion increased from 6.7% in 2017 to 13% in 2021, among 17 to 19 year olds the proportion increased from 44.6% in 2017 to 58.2%.

Whilst there is a growing body of evidence that suggests correlations between poorer mental health and wellbeing among some children and young people over the pandemic and restrictions, the nature of the research makes it difficult to establish a causal link between the 2 factors.

Evidence from across the pandemic suggests that most children had broadly coped well with the pandemic (https://www.childrenssociety.org.uk/information/professionals/resources/good-childhood-report-2021). In their Big Ask survey (https://www.childrenscommissioner.gov.uk/report/the-big-ask-big-answers/) in April and May 2021 - a survey of over a half a million children and young people aged between 6 and 17 years - the Children's Commissioner found that 80% of children and young people were happy or okay with their mental wellbeing. However, over the pandemic, girls and young women, older young people (16 to 24 year olds), disadvantaged children and young people, and those with SEND were more likely to report difficulties with mental health and wellbeing (references 4, 6 to 11, 15, 16, 17, 24, 29, 30, 31, 34, 36). However, the methodology of the literature is unable to determine if these are continuations of pre-pandemic trends or reflect differential experiences between groups of children through the pandemic.

Finally, wellbeing scores between March and July 2021 appeared lower than during a similar period of reducing restrictions in the previous year. (references 4, 12) It is unclear from the current research why this may be the case.

Special educational needs (and disability) (SEND)

Some children with SEND appear to have suffered substantial impacts on their mental health and experienced greater anxiousness during the pandemic than children without SEND.

There was evidence of increasing behavioural and emotional difficulties, as well as mental health problems, in children and young people with SEND, in the first year of the pandemic (https://www.familyfund.org.uk/the-impact-of-covid-19-a-year-in-the-life-of-families-raising-disabled-children). Throughout the pandemic up to June 2021, parents/carers of children with SEND reported that their children experienced higher rates of mental health symptoms and greater anxiety than parents/carers of children without SEND (references 4, 6, 30, 31). Evidence from the period August 2020 to July 2021 suggested that for other subjective wellbeing measures (such as happiness, life satisfaction, and feeling life to be worthwhile) for secondary aged children with SEND were similar to those without SEND (https://www.gov.uk/government/publications/state-of-the-nation-2021-children-and-young-peoples-wellbeing). Similar to patterns seen pre pandemic, children and young people with SEND were also more likely to have poorer mental health than those without SEND. The State of the Nation (https://www.gov.uk/government/publications/state-of-the-nation-2021-children-and-young-peoples-wellbeing) also reported that trends by SEND status were not clear or consistent.

When asked about their child's wellbeing, parents/carers of children with SEND reported that their children had high anxiety, were socially isolated, and were unhappy in the January to March 2021 lockdown (references 30 to 32). In January 2021, a higher proportion of children with SEND than children without were identified as having possible/probable mental disorders (references 11, 29). Parents/carers of children with SEND also reported that elevated levels of behavioural and emotional difficulties were sustained between February and April 2021 (http://cospaceoxford.org/findings/changes-in-childrens-mental-health-symptoms-from-march-2020-to-june-2021/) whereas children without SEND saw declines in these difficulties during the same period.

However, the lockdown period from January to March 2021 appeared to have been beneficial to some pupils with SEND. Some parents/carers reported that their children were better motivated, engaged and were responding well to working flexibly and independently (https://www.familyfund.org.uk/the-impact-of-covid-19-a-year-in-the-life-of-families-raising-disabled-children). Moreover, some parents/carers stated that their child was less stressed during lockdown because they had not been to school and had been enjoying spending more time with their parents/carers (references 30, 32). In addition, staff at special schools and colleges also reported that some students who had attended their educational setting throughout the pandemic had benefited from changes in provision such as smaller groups, quieter learning environments, and more one to one input (https://www.nfer.ac.uk/special-schools-and-colleges-experiences-of-the-covid-19-pandemic-summer-2021/).

After the lockdown, in the 2021 summer term, staff and parents of pupils attending special schools or colleges reported in interviews poorer pupil wellbeing - particularly greater anxiety - and declines in student mental health. Staff and parents reported that these poorer outcomes were a result of the pandemic and restrictions. Staff felt that students were 5 to 8 months behind in their emotional health and mental wellbeing compared to pre-pandemic expectations for each pupil's development. Parents and staff also reported deterioration in behavioural, social, and independent skills in some pupils (https://www.nfer.ac.uk/special-schools-and-colleges-experiences-of-the-covid-19-pandemic-summer-2021/). Children and young people with SEND were also found to have poorer wellbeing and greater anxiety in the 2021 autumn term (https://impacted.org.uk/impactinpractice) than their peers without SEND, as well as having more difficulty thinking about how they learn and lower learning engagement.

Gender and age

Data from across the pandemic indicates that boys and girls have had different mental health and wellbeing challenges.

Evidence found that, throughout the pandemic (up to June 2021), parents/carers of school aged children reported higher symptoms of behavioural and attentional difficulties for boys than girls. However, girls had higher levels of emotional difficulties (http://cospaceoxford.org/findings/changes-in-childrens-mental-health-symptoms-from-march-2020-to-june-2021/). Young women (aged 16 to 24) were more likely to report greater psychological distress than young men (https://pubmed.ncbi.nlm.nih.gov/34716130/) and girls were also found to have substantially poorer wellbeing, on average, and greater anxiety during the pandemic than boys (references 8, 16). In April and May 2021, girls were twice as likely to report that they were unhappy with their mental health compared to boys. Teenage girls (16 to17) were also most worried about their mental wellbeing (https://www.childrenscommissioner.gov.uk/report/the-big-ask-big-answers/) than other groups of children and young people.

Differences in rates of mental disorder between boys and girls also appeared to change with age. Among 6 to 10 year olds, boys were nearly twice as likely to have a probable mental disorder than girls, whilst among young people (aged 17 to 23) this gender difference had reversed with prevalence of mental disorder in young women being more than double that in young men (https://digital.nhs.uk/data-and-information/publications/statistical/mental-health-of-children-and-young-people-in-england/2021-follow-up-to-the-2017-survey). There was no difference in rates of probable mental disorder between 11 to16 year old girls and boys.

While these gender differences are fairly typical (https://digital.nhs.uk/data-and-information/publications/statistical/mental-health-of-children-and-young-people-in-england/2017/2017), observed differences between males and females appear to have increased during the pandemic (references 2, 6, 8, 9). However, it is unclear from the literature what caused the widening of this gap.

Disadvantaged Children and Young People

There is evidence that, over the pandemic, disadvantaged children and young people have had poorer mental health and wellbeing outcomes than those with more advantage (references 10, 15, 16, 19, 24, 31, 34).

In 2020, young people aged 16 to 24 who lived in the most deprived areas of the UK experienced increases in psychological distress 3.4 times larger (https://pubmed.ncbi.nlm.nih.gov/34716130/) than those in the least deprived areas. There was also evidence that children with a probable mental disorder (https://digital.nhs.uk/data-and-information/publications/statistical/mental-health-of-children-and-young-people-in-england/2020-wave-1-follow-up) were more likely to live in a household that had fallen behind with payments such as bills, rent, and mortgage.

Throughout the pandemic, parents in households with lower annual incomes reported their children had more symptoms of behavioural, emotional, and attentional difficulties than those with higher annual income (references 4, 5). Between February and April 2021, parents/carers of children from higher income households reported that these symptoms decreased in their children as lockdown eased. However, there was no statistical change in these symptoms for those from lower income households (http://cospaceoxford.org/findings/changes-in-childrens-mental-health-symptoms-from-march-2020-to-june-2021/) in this period.

Pre-existing mental health needs

There is some evidence that indicates a range of challenges and experiences for children and young people with pre-existing mental health needs, particularly on accessing support. A study using 2 waves of data collected between January 2018 and March 2019, and May 2020 (https://www.sciencedirect.com/science/article/pii/S0278584621000890?via%3Dihub), found that young people (aged 17-19) identified as having emotional difficulties or high symptom levels pre-pandemic also reported experiencing higher levels of stress, conflict, loneliness, and lower levels of perceived social support, than young people without emotional difficulties or lower symptoms early in the pandemic.

However, among adolescents aged 10 to 16 years old, there was evidence that in July 2020 those with higher levels of mental health problems before the pandemic experienced improvements in their mental health, whilst those with prior lower levels of problems experienced increases in mental health problems (https://www.jahonline.org/article/S1054-139X(21)00198-1/fulltext). Other research compared mother and child reporting of young adolescent depression and behaviour problems between June-August 2020 to immediately pre-pandemic. It found that, according to mother's reports of young adolescent depression and behaviour problems, the COVID-19 pandemic appeared to have had a greater effect on those not previously identified with higher mental health symptoms (https://acamh.onlinelibrary.wiley.com/doi/10.1111/jcv2.12008) than adolescents with previous mental health problems. This difference was not found in adolescent self-reporting.

Among some secondary aged pupils that had struggled with pre-existing mental health issues during the lockdown (March to July 2020), this continued into the return to school in Autumn 2020 (references 27, 29 to 37). There is also qualitative evidence that the stress around going back to school was a trigger for some young people (https://learning.nspcc.org.uk/research-resources/2020/coronavirus-insight-briefing-schools) who started to self-harm again or have suicidal thoughts. Similarly, in analysis of social media discussions posted between March 2020 and March 2021, some young people expressed worries about resurfacing mental health problems, linking this to their return to school (https://pubmed.ncbi.nlm.nih.gov/35010612/).

Children and young people with pre-existing mental health conditions also indicated challenges with accessing support for their mental health needs. Data collected between January and February 2021 reported that almost half of those who felt they needed mental health support during the pandemic had either not accessed or not looked for any support.

Barriers to seeking support included concerns about seeking help - for example, being aware of how many people were struggling and worrying about overstretched services. Respondents also cited concerns about what family and friends might think if they sought support, how it was hard to seek support without others finding out and that they felt too busy due to education or work pressures.

There were also barriers around the types of support available, such as concerns about only having access to online or telephone support. There was also reporting of barriers such as long waiting lists, being discouraged from receiving further support from GPs because of waiting lists and high thresholds, and lack of money to fund private support when respondents felt they would not be able to access NHS support (references 35 to 38).

Black, Asian, and minority ethnic (BAME)

There is inconsistent evidence that pandemic impacts on mental health and wellbeing differed by ethnicity. This may be due to small sample sizes and the combining of ethnic groups potentially masking differences between them.

There is some evidence that there were greater increases in psychological distress among white children and Indian children during the pandemic (https://pubmed.ncbi.nlm.nih.gov/34716130/) compared to Pakistani and Bangladeshi, black Caribbean, African, and other ethnicities. White British and mixed ethnicity children and young people were also more likely to show symptoms of probable mental disorder (https://digital.nhs.uk/data-and-information/publications/statistical/mental-health-of-children-and-young-people-in-england/2021-follow-up-to-the-2017-survey). However, other evidence suggests that children and young people from black, Asian and minority ethnic (BAME) backgrounds have experienced a higher rate of mental health and wellbeing concerns during the pandemic (references 17 to 22).

Some studies have not found differences in overall psychological wellbeing, subjective wellbeing, and difficulties for children by ethnic groups (references 14, 19, 39).

Lesbian, gay, bisexual, and transgender (LGBT+)

Overall, little evidence was identified that met the inclusion criteria for this report that documented the experiences of England's LGBT+ children and young people over the pandemic.

In a study conducted between December 2020 and January 2021 (https://www.justlikeus.org/single-post/growing-up-lgbt-just-like-us-research-report), a greater proportion of LGTB+ respondents (aged 11 to 18) reported that their mental health had worsened since the start of the pandemic, compared to non LGBT+ respondents. LGBT+ respondents were also more likely to report mental health challenges such as anxiety disorder, depression and panic attacks, and suicidal thoughts and feelings.

Without a pre-pandemic baseline for comparison, it is not possible to know if the greater reporting of mental health challenges by LGBT+ respondents is an indication of specific pandemic impacts, or a continuation of pre-pandemic patterns.

The study also found that LGBT+ respondents have experienced feeling lonely/separated from people and experienced tension in the place they live more than non-LGBT+ respondents during the lockdown restrictions.

Risks and experiences

Family relationships

In a study conducted between August and October 2020, primary school pupils reported generally feeling that their family relationships were going well, more so than secondary school pupils. They felt that lockdown had positively impacted their family relationships because they got to spend more time together (references 34, 41, 42). Parents also reported this throughout the lockdown period and into the autumn term (references 24, 43, 44). Some pupils reported that the lockdown helped communication with some of their family members as they were able to have deeper conversations about mental health and reflect on shared challenges. Between April and May 2021, the Big Ask survey found that most children and young people aged 6 to 17 years old were happy or okay with their families (https://www.childrenscommissioner.gov.uk/report/the-big-ask-big-answers/).

However, there is some evidence that extended periods of time spent with family without relief during lockdown led to increased conflict between children with SEN and their siblings. Children with SEN were reportedly more likely to be picked on by their siblings (https://bpspsychub.onlinelibrary.wiley.com/doi/10.1111/bjep.12451) in June 2020, the third month of lockdown, than any other time between March and October 2020.

Survey evidence from March to May 2020 found that some young people (aged 11 to 16) who had a closer relationship with their parents reported significantly less severe symptoms of mental health difficulties and lower levels of loneliness (https://www.sciencedirect.com/science/article/pii/S0165032721003402?via%3Dihub). However, children and young people with a probable mental disorder were more likely to also have a parent with a higher level of psychological distress (https://digital.nhs.uk/data-and-information/publications/statistical/mental-health-of-children-and-young-people-in-england/2020-wave-1-follow-up) and these rates were higher than seen in 2017. During the pandemic, caregivers with higher psychological distress were significantly more likely to report that their children experienced psychological distress (https://www.revistapcna.com/), were more likely to argue with the rest of the family and be more dependent on their caregiver. Other research similarly found that parent/carers with higher psychological distress were more likely to report that their child had greater emotional symptoms, conduct problems, and attentional difficulties (https://acamh.onlinelibrary.wiley.com/doi/10.1111/jcpp.13490) than parents/carers with fewer symptoms of psychological distress.

The link between parent's mental health and children's mental health is well established (https://link.springer.com/article/10.1007/s00127-018-1604-0) and it is not always clear which factor causes the other. Throughout the pandemic, parents/carers from single adult households reported higher levels of all behavioural, emotional, and restless/attentional symptoms (https://cospaceoxford.org/findings/changes-in-children-mental-health-symptoms-from-march-2020-to-jan-2021/) than parents/carers from non-single parent households, as did those who reported having prenatal depression (https://acamh.onlinelibrary.wiley.com/doi/10.1111/jcv2.12008). During April to May 2020, parents from the most disadvantaged families and those with children who had SEND were more likely to have experienced mental distress (https://www.gov.uk/government/publications/early-education-and-development-coronavirus-covid-19-study). In their own online discussions during lockdowns, some young people explicitly linked fraught or abusive family relationships to their depression, anxiety, self-harm, and suicidal thoughts (https://pubmed.ncbi.nlm.nih.gov/35010612/).

Further evidence on parent and carer mental health during the pandemic can be accessed in this spotlight report (https://www.gov.uk/government/publications/covid-19-mental-health-and-wellbeing-surveillance-spotlights/parents-and-carers-spotlight) and important findings relevant to parent mental health are also included in the Important findings chapter (https://www.gov.uk/government/publications/covid-19-mental-health-and-wellbeing-surveillance-report/2-important-findings-so-far) of this surveillance report.

Loneliness

There is some evidence that young people (11 to 16 years old) who have reported higher loneliness had significantly higher symptoms of mental health difficulties (https://www.sciencedirect.com/science/article/pii/S0165032721003402?via%3Dihub) during the first lockdown. Some studies have reported that between March and November 2020, there was a lack of social connectedness, mainly for primary school children and those children without access to the internet (references 27, 41, 49). However, other research has found that it was secondary pupils that were more likely to report feeling lonely than primary aged pupils (https://psyarxiv.com/gtbfm/). Even after returning to school, some children and young people found it hard to reconnect with friendship groups (references 27, 41, 49).

Pupils of both primary and secondary age have also felt like they sometimes do not have anyone to talk to or have felt left out (https://learning.nspcc.org.uk/research-resources/2020/coronavirus-insight-briefing-schools). Other evidence further suggests that it is more common for children and young people with a probable mental disorder to feel lonely (references 11, 15). However, there is no comparison for this prior to the pandemic and no causal link can be established from the literature.

Anxiousness

From the first lockdown and during the return to school in September 2020, there was an increase in reported anxiety and stress for some children who were worried about contracting and spreading COVID-19 (references 3, 24, 27, 42, 49). This was especially felt by secondary pupils who had vulnerable family members at home (https://digital.nhs.uk/data-and-information/publications/statistical/mental-health-of-children-and-young-people-in-england/2020-wave-1-follow-up). Some children and young people were worried and anxious about catching up with their schoolwork and uncertainty about the plans for exams (references 41, 49).

Another activity which increased anxiety around COVID-19 was reading or watching the news and social media (references 41, 49).

Mental health services

Some children and young people have reported that accessing counselling at school was more difficult after the first lockdown (https://learning.nspcc.org.uk/research-resources/2020/coronavirus-insight-briefing-schools) due to long waiting lists, being unable to attend sessions if they were self-isolating, and not being able to have regular appointments.

Also see the evidence summarised above (https://www.gov.uk/government/publications/covid-19-mental-health-and-wellbeing-surveillance-report/7-children-and-young-people#Pre-existing) for children and young people with existing mental health needs reporting challenges with accessing support.

For data on children and young people's use of NHS mental health services and Eating disorder services, see published data from the Mental Health Services Data Set and CYP Eating Disorder Statistics (https://www.england.nhs.uk/statistics/statistical-work-areas/cyped-waiting-times/).

Digital exclusion and workspaces

There is evidence that, compared to the least disadvantaged, children in the most disadvantaged group were less likely to have access to a computer during periods of lockdown (https://www.gov.uk/government/publications/early-education-and-development-coronavirus-covid-19-study). In the early stages of the pandemic, mental health symptoms were found to have increased more in children who did not have access to a computer (https://www.medrxiv.org/content/10.1101/2021.11.25.21266853v1?ct=) for home schooling during school closures compared to children who did have access to a computer. Similarly, children on pupil premium were less likely to have a quiet place to study (https://impacted.org.uk/impactinpractice) and not having this space was linked to lower outcomes in wellbeing.

Attendance

In the Department for Education's most recent State of the Nation report (https://www.gov.uk/government/publications/state-of-the-nation-2021-children-and-young-peoples-wellbeing), new analysis was presented on relationships between school attendance and pupil wellbeing. It found that there was a consistent link throughout the year between pupil's wellbeing and how frequently they attended school. Pupils with higher happiness scores were more likely to have attended school every or most days than pupils with lower scores.

When examining pupils' experiences after the return to school in October 2020 after lockdown, the report also found a relationship between experiences at school and wellbeing. Pupils with higher happiness ratings reported that they felt happier about returning to school, were better able to concentrate, and were less concerned about catching up than pupils with lower happiness ratings. Pupils with higher anxiousness ratings found it harder to concentrate, were more worried about catching up, and were less likely to report being happy to have returned to school.

Importantly, for both discussed relationships, this analysis could not identify causal links and presented relationships may be explained by other factors.

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